



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST – 8 PAPER -2 TEST DATED ON 20th JUNE 2026

Q1.Regarding the pathophysiology, tumor biology, and metastatic behavior of this malignancy, all of the following statements are true EXCEPT:

- a) Lymphatic spread preferentially follows the gonadal vessel drainage pattern to para-aortic lymph nodes, reflecting embryologic origin
- b) Elevated AFP levels arise due to differentiation toward extraembryonic yolk sac structures, recapitulating fetal protein synthesis
- c) Overexpression of OCT3/4 and NANOG indicates loss of pluripotency and terminal differentiation of tumor cells
- d) Isochromosome 12p contributes to tumorigenesis by promoting genomic instability and proliferation of germ cell neoplasia in situ–derived cells

Q2.66-year-old man presents with progressively worsening lower urinary tract symptoms over 8 months, including hesitancy, weak stream, and nocturia. He denies hematuria or weight loss. There is no prior history of prostate cancer.

Investigations:

- PSA: 9.2 ng/mL
- Free/total PSA ratio: 9%
- Digital rectal examination: firm area in the right posterior prostate, no obvious extracapsular fixation

A multiparametric MRI of the prostate reveals:A 1.4 cm lesion in the peripheral zone (right posterolateral mid-gland) .The lesion is markedly hypointense on T2-weighted imaging .Shows restricted diffusion on DWI/ADC sequences .Demonstrates early focal enhancement on dynamic contrast imaging .The lesion is assigned a PI-RADS score of 5.

What is the most appropriate next step in management?

- a) Repeat PSA testing after 6 months
- b) Initiate 5-alpha reductase inhibitors
- c) Perform MRI-targeted transrectal or transperineal prostate biopsy
- d) Proceed directly to radical prostatectomy

Q3.Regarding the pathophysiology, molecular biology, and clinical behavior of this malignancy, all of the following statements are true EXCEPT:

- a) Loss of function of the VHL gene results in accumulation of hypoxia-inducible factors, promoting angiogenesis via increased VEGF expression



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- b) The tumor commonly arises from proximal tubular epithelial cells and is characterized by lipid- and glycogen-rich clear cytoplasm
- c) Hematogenous spread frequently involves the lungs and bones, facilitated by early invasion of the renal vein
- d) The tumor typically demonstrates central stellate scarring on imaging, which helps distinguish it from malignant lesions

Q4. Based on the clinical, radiological, histopathological, and molecular findings, all of the following statements are true EXCEPT:

- a) In this dialysis-dependent patient with ESRD and a hypovascular, homogeneous renal mass, the tumor is likely papillary RCC, which is characteristically associated with acquired cystic kidney disease and often demonstrates trisomy 7 and 17
- b) The presence of papillary fronds with fibrovascular cores and intratumoral foamy macrophages supports the diagnosis of papillary RCC, and activation of the MET proto-oncogene pathway plays a key role in its tumorigenesis
- c) Despite the CT findings of relatively low enhancement, papillary RCC is typically a hypervascular tumor with intense arterial phase enhancement similar to clear cell RCC due to increased angiogenic signaling
- d) In contrast to clear cell RCC, the absence of renal vein invasion and the relatively indolent clinical presentation in this patient are consistent with the usual biological behavior of papillary RCC

Q5. Based on the clinical, radiological, histopathological, and genetic findings, all of the following statements are true EXCEPT:

- a) In this patient with fibrofolliculomas and family history of pneumothorax suggestive of Birt-Hogg-Dubé syndrome, the presence of a well-circumscribed, homogeneous renal mass composed of polygonal cells with perinuclear halos supports chromophobe RCC arising from intercalated cells of the collecting duct
- b) The imaging findings of a homogeneous renal mass with mild-to-moderate enhancement and absence of necrosis or renal vein invasion in this asymptomatic patient are consistent with the relatively indolent biological behavior of chromophobe RCC
- c) The histopathological features described, including pale eosinophilic cytoplasm, prominent cell membranes, and perinuclear halos, along with cytokeratin 7 positivity, are characteristic of chromophobe RCC and reflect underlying cytoplasmic vesicle accumulation and mitochondrial alterations



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- d) In this patient with a well-defined renal mass showing homogeneous enhancement on CT, chromophobe RCC is expected to be highly hypervascular with intense arterial phase enhancement due to VEGF-mediated angiogenesis similar to clear cell RCC

Q6. A 32-year-old woman presents with recurrent episodes of gross hematuria and bilateral flank pain over the past 2 years. She has had three episodes of culture-positive urinary tract infection in the last 18 months. She denies any history of nephrolithiasis or prior renal surgery. Her blood pressure is 118/76 mm Hg. Urinalysis shows microscopic hematuria and mild proteinuria. Serum creatinine is 0.9 mg/dL. Renal ultrasound reveals multiple small echogenic foci in the medullary pyramids bilaterally without hydronephrosis. Non-contrast CT shows punctate calcifications arranged in a linear, brush-like pattern within the medulla. Which of the following is the most likely underlying anatomical abnormality in this patient?

- a) Dilatation of the collecting ducts of Bellini within the medullary pyramids
- b) Cystic dilatation of Bowman's capsule in the renal cortex
- c) Atrophy of the medullary interstitium with sparing of collecting ducts
- d) Fusion of the lower poles of both kidneys across the midline

Q7. A 32-year-old woman with long-standing neurogenic bladder due to spinal cord injury presents with low bladder capacity, high intravesical pressures, recurrent urinary tract infections, and worsening hydronephrosis despite maximal medical therapy; urodynamic studies show poor compliance. Which of the following is the most appropriate surgical management to preserve renal function?

- a) Ileal conduit urinary diversion
- b) Augmentation cystoplasty using ileal segment
- c) Radical cystectomy
- d) Ureteroneocystostomy

Q8. A full-term male neonate is noted to have bilateral renal agenesis on postnatal ultrasound after presenting with respiratory distress and characteristic facial features, including flattened nose and low-set ears. Pregnancy history reveals severe oligohydramnios.

Further evaluation suggests failure of a critical embryologic interaction necessary for kidney development.

Which of the following embryologic defects most likely accounts for this condition?

- a) Failure of the ureteric bud to interact with the metanephric mesenchyme
- b) Failure of ascent of the metanephric kidneys from the pelvis



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- c) Persistence of the mesonephric duct without regression
- d) Abnormal differentiation of the pronephros during early development

Q9. The ureteric bud is derived from:

- a) Paramesonephric duct
- b) Mesonephric duct
- c) Metanephric mesenchyme
- d) Cloaca

Q10. Nephrons are derived from:

- a) Ureteric bud
- b) Mesonephric duct
- c) Metanephric mesenchyme
- d) Cloacal membrane

Q11. Failure of kidney ascent results in:

- a) Multicystic dysplastic kidney
- b) Pelvic kidney
- c) Horseshoe kidney
- d) Renal agenesis

Q12. A fetus with bilateral renal agenesis most likely has a defect in which signaling interaction?

- a) GDNF–RET signaling between metanephric mesenchyme and ureteric bud
- b) SHH signaling in cloacal partitioning
- c) BMP4-mediated apoptosis of mesonephric duct
- d) WNT7a signaling in Müllerian duct development

Q13. A duplicated ureter draining a single kidney most likely results from:

- a) Two separate ureteric buds arising from mesonephric duct
- b) Failure of ureteric bud apoptosis
- c) Abnormal ascent of metanephric kidney
- d) Persistence of pronephric duct



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Q14. In a horseshoe kidney, ascent is limited due to entrapment beneath:

- a) Superior mesenteric artery
- b) Inferior mesenteric artery
- c) Common iliac artery
- d) Median sacral artery

Q15. Failure of incorporation of mesonephric ducts into the posterior bladder wall results in abnormal development of:

- a) Bladder dome
- b) Trigone
- c) Urachus
- d) Detrusor muscle

Q16. A mutation impairing ureteric bud branching would most directly affect development of:

- a) Glomeruli
- b) Proximal tubules
- c) Collecting ducts
- d) Bowman's capsule

Q17. Ectopic ureter insertion into the vagina is most commonly due to:

- a) Delayed ureteric bud formation
- b) Cranial origin of ureteric bud on mesonephric duct
- c) Caudal origin of ureteric bud on mesonephric duct
- d) Failure of metanephric differentiation

Q18. Which embryologic error explains multicystic dysplastic kidney?

- a) Excess ureteric bud branching with normal induction
- b) Failure of ureteric bud to induce organized nephron formation
- c) Failure of mesonephric duct regression
- d) Persistence of cloacal membrane



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- Q19. The metanephric kidney begins to function at which developmental stage?
- 5th week
 - 7th week
 - 9th–10th week
 - After birth only
- Q20. A 28-year-old man presents with acute colicky pain radiating from flank to groin. Non-contrast CT shows a 5 mm calculus at the level of the sacroiliac joint where the ureter crosses the pelvic brim. Which anatomical and vascular relationship explains this site of impaction?
- Compression between renal pelvis and psoas muscle
 - Crossing anterior to the common iliac vessels
 - Oblique intramural course through bladder wall
 - Passage posterior to gonadal vessels
- Q21. During laparoscopic hysterectomy, a surgeon ligates the uterine artery near the cervix. Postoperatively, the patient develops flank pain and hydronephrosis. Which constriction site is most likely injured?
- Pelviureteric junction
 - Crossing over iliac vessels
 - Ureterovesical junction near intramural segment
 - Upper ureter near renal hilum
- Q22. A radiologist identifies a ureteric calculus at the narrowest segment of the ureter that also functions as a physiological anti-reflux mechanism. Which structural feature is responsible for this dual function?
- High-pressure gradient at PUJ
 - Compression by iliac vessels
 - Oblique intramural tunnel within bladder wall
 - Peristaltic failure at mid ureter
- Q23. A 40-year-old male has recurrent stones lodging at different ureteric sites. Which of the following correctly matches anatomical level with vertebral level?
- PUJ – L4



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- b) Iliac crossing – L1
- c) UVJ – pelvic cavity below sacroiliac joint
- d) Intramural ureter – T12

Q24. A ureteric stone is visualized just distal to the renal pelvis where the ureter narrows due to transition from funnel-shaped pelvis to tubular ureter. Which factor primarily contributes to this constriction?

- a) Vascular compression
- b) Change in luminal diameter and muscle arrangement
- c) Oblique bladder wall entry
- d) External ligamentous fixation

Q25. A stone lodged at the second ureteric constriction is most likely to produce referred pain to:

- a) Epigastrium
- b) Periumbilical region
- c) Groin and scrotum/labia
- d) Suprapubic region only

Q26. Which of the following best explains why the ureterovesical junction is the most common site of stone impaction?

- a) It has the least peristaltic activity
- b) It is the longest segment of ureter
- c) It is the narrowest anatomical and functional segment
- d) It lacks smooth muscle

Q27. During ureteroscopy, the surgeon notes resistance at three predictable points. Which sequence is correct from proximal to distal?

- a) UVJ → iliac crossing → PUJ
- b) PUJ → iliac crossing → UVJ
- c) Iliac crossing → PUJ → UVJ
- d) PUJ → UVJ → iliac crossing



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- Q28. A 42-year-old man with hypertension and bilateral enlarged cystic kidneys develops sudden severe headache and vomiting. CT head shows hemorrhage.
Which associated anomaly most likely explains this presentation?
- a) Berry aneurysm of the circle of Willis
 - b) Arteriovenous malformation
 - c) Cavernous hemangioma
 - d) Dural venous sinus thrombosis
- Q29. A patient with known PKD presents with early satiety and abdominal fullness. Imaging reveals multiple cysts in another organ apart from kidneys. Which is the most common extrarenal manifestation?
- a) Pancreatic cysts
 - b) Hepatic cysts
 - c) Splenic cysts
 - d) Adrenal cysts
- Q30. A 35-year-old woman with PKD develops a holosystolic murmur best heard at the apex radiating to axilla.
Which associated abnormality is most likely present?
- a) Aortic stenosis
 - b) Mitral valve prolapse
 - c) Tricuspid regurgitation
 - d) Pulmonary stenosis
- Q31. A neonate presents with massively enlarged kidneys and respiratory distress due to pulmonary hypoplasia.
Which type of PKD is most likely?
- a) Autosomal dominant PKD
 - b) Autosomal recessive PKD
 - c) Medullary sponge kidney
 - d) Multicystic dysplastic kidney
- Q32. A child with PKD develops portal hypertension and hepatosplenomegaly.
Which associated pathology explains this finding?



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- a) Hepatic cyst rupture
- b) Congenital hepatic fibrosis
- c) Cirrhosis due to alcoholic mom
- d) Budd–Chiari syndrome

Q33. A patient with ADPKD is screened for a life-threatening vascular complication. Which investigation is most appropriate?

- a) Echocardiography
- b) CT coronary angiography
- c) MR angiography of cerebral vessels
- d) Doppler renal artery

Q34. A 34-year-old woman with known horseshoe kidney undergoes CT angiography prior to abdominal surgery. Imaging demonstrates multiple aberrant renal arteries arising from the aorta and iliac vessels supplying the renal isthmus and poles.

What is the most accurate explanation for this vascular pattern?

- a) Compensatory angiogenesis due to chronic renal hypoxia
- b) Persistence of embryonic segmental arteries due to arrested ascent
- c) Fusion-induced neovascularization of renal parenchyma
- d) Abnormal branching of ureteric bud leading to vascular anomalies

Q35. A 10-year-old girl with short stature, webbed neck, and primary amenorrhea is found to have a horseshoe kidney on imaging. Genetic testing confirms monosomy X.

Which additional renal or vascular anomaly is most likely in this patient?

- a) Bilateral renal agenesis
- b) Multiple aberrant renal arteries with malrotation
- c) Duplex ureter with vesicoureteral reflux
- d) Medullary sponge kidney

Q36. A 40-year-old man presents with hematuria and is found to have a horseshoe kidney. CT scan reveals a solid renal mass arising near the isthmus.

Which malignancy is most strongly associated with this congenital anomaly?

- a) Clear cell renal cell carcinoma
- b) Transitional cell carcinoma



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- c) Wilms tumor
- d) Oncocytoma

Q37. A radiological study of a patient with horseshoe kidney shows anteriorly directed renal pelvises and calyces.

This abnormal orientation most likely results from which developmental defect?

- a) Failure of lateral rotation of kidney
- b) Incomplete medial rotation during ascent
- c) Excessive cranial migration
- d) Absence of ureteric bud branching

Q38. A 30-year-old patient with horseshoe kidney presents with recurrent nephrolithiasis. Imaging reveals urinary stasis within the collecting system.

Which anatomical feature most significantly contributes to this predisposition?

- a) Increased renal perfusion pressure
- b) Abnormal ureteral insertion causing vesicoureteral reflux
- c) Malrotation and high ureteral insertion causing impaired drainage
- d) Reduced nephron number

Q39. A 7-year-old girl presents with recurrent febrile urinary tract infections and intermittent urinary incontinence despite normal voiding habits. Ultrasound shows a duplex collecting system on the left side. Voiding cystourethrogram reveals vesicoureteral reflux into the lower pole ureter, while MRI shows hydronephrosis of the upper pole moiety with ureterocele formation.

Which embryologic abnormality best explains this pattern?

- a) Early bifurcation of a single ureteric bud before entering metanephric mesenchyme
- b) Formation of two separate ureteric buds from the mesonephric duct
- c) Failure of ureteric bud to canalize
- d) Persistence of pronephric duct

Q40. A 5-year-old girl presents with continuous dribbling of urine despite normal voluntary voiding. Imaging reveals a duplicated collecting system with the upper pole ureter inserting ectopically into the vagina.

Which anatomical principle best explains this finding?

- a) Upper pole ureter inserts superior and lateral in bladder



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- b) Lower pole ureter inserts inferior and medial
- c) Upper pole ureter inserts inferior and medial relative to lower pole ureter
- d) Both ureters insert at the same level

Q41. A 40-year-old man undergoing evaluation for flank pain is found to have a bifid ureter that joins before entering the bladder. There is no reflux or obstruction. Which embryologic mechanism most likely caused this anomaly?

- a) Two independent ureteric buds
- b) Early splitting of a single ureteric bud
- c) Failure of ureteric bud induction
- d) Abnormal cloacal partitioning

Q42. A patient with duplicated ureter develops hydronephrosis of the upper pole with ureterocele formation at the bladder. Which mechanism explains ureterocele formation?

- a) Increased glomerular filtration
- b) Obstruction at ectopic distal ureteral orifice
- c) Hyperplasia of bladder muscle
- d) Vesicoureteral reflux

Q43. A neonate is diagnosed with duplex kidney. Which complication is most likely to affect the upper pole moiety?

- a) Vesicoureteral reflux
- b) Ureterocele and obstruction
- c) Renal artery stenosis
- d) Glomerulonephritis

Q44. A patient with duplicated ureter has one ureter inserting into the urethra distal to the sphincter. Which symptom is most characteristic?

- a) Intermittent hematuria
- b) Continuous urinary dribbling
- c) Acute urinary retention
- d) Flank mass only



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- Q45. A patient with duplicated ureter undergoes surgery. Which ureter is more likely to be obstructed?
- Lower pole ureter
 - Upper pole ureter
 - Both equally
 - Neither
- Q46. A duplicated ureter anomaly is suspected. Which imaging modality best demonstrates reflux and anatomy simultaneously?
- Ultrasound
 - Non-contrast CT
 - Voiding cystourethrogram
 - Plain X-ray
- Q47. Under the National Immunization Schedule in India, which vaccine is routinely administered to a newborn at birth?
- DPT
 - BCG
 - Measles-rubella
 - Pentavalent vaccine
- Q48. At what age does a normally developing infant usually sit without support?
- 3 months
 - 6 months
 - 9 months
 - 12 months
- Q49. A 9-month-old infant has high fever for 4 days. After the fever subsides, a generalized rash appears, but the child remains active. Which virus is the most likely cause?
- Parvovirus B19
 - Human herpesvirus 6
 - Varicella-zoster virus



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d) Coxsackie virus

Q50. A 2-year-old child presents with fever, cough, conjunctivitis, Koplik spots, and a maculopapular rash beginning behind the ears. What is the most likely diagnosis?

- a) Rubella
- b) Measles
- c) Roseola infantum
- d) Scarlet fever

Q51. Regarding thyroglossal duct cyst, all are true EXCEPT:

- a) It usually presents as a midline neck swelling.
- b) It moves upward with swallowing and protrusion of the tongue.
- c) Definitive treatment is Sistrunk operation, including removal of the central part of the hyoid bone.
- d) It is a lateral neck swelling at the anterior border of sternocleidomastoid and does not move with tongue protrusion.

Q52. Regarding Ludwig angina, all are true EXCEPT:

- a) It is a rapidly spreading cellulitis involving submandibular, sublingual, and submental spaces.
- b) Dental infection is a common source.
- c) It may cause brawny floor-of-mouth swelling, tongue elevation, dysphagia, drooling, and airway compromise.
- d) It is a localized benign tonsillar crypt infection with no risk of airway obstruction.

Q53. Regarding retropharyngeal abscess, all are true EXCEPT:

- a) It is more common in young children due to retropharyngeal lymph nodes that may suppurate after infection.
- b) It may present with fever, neck stiffness, dysphagia, drooling, muffled voice, and respiratory distress.
- c) It can spread into the mediastinum and may become life-threatening.
- d) It is a benign accumulation of tonsillar debris inside crypts and has no airway or mediastinal risk.

Q54. Which of the following statements regarding fetal growth and developmental chronology is INCORRECT?



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- a) Neural crest cell migration and neural tube closure are largely complete by the end of the 4th week, and disruption during this period leads to major structural anomalies rather than functional deficits
- b) Type II pneumocytes begin surfactant synthesis around 20–24 weeks, but phosphatidylglycerol production and structurally mature lamellar bodies typically appear later, correlating with improved neonatal survival beyond 28 weeks
- c) Nephrogenesis continues until approximately 34–36 weeks, and premature interruption of this process is associated with long-term reduction in nephron number
- d) Hepatic hematopoiesis becomes negligible by the end of the first trimester, with bone marrow assuming the dominant hematopoietic role by 12–14 weeks

Q55. A 7-year-old child develops cola-colored urine, facial puffiness, and high blood pressure 2 weeks after a sore throat. What is the most likely diagnosis?

- a) Minimal change disease
- b) Acute post-streptococcal glomerulonephritis
- c) IgA nephropathy
- d) Nephrotic syndrome

Q56. A child develops pallor, petechiae, bone pain, and recurrent fever. Peripheral blood smear shows blast cells. What is the most likely diagnosis?

- a) Aplastic anemia
- b) Acute leukemia
- c) Iron deficiency anemia
- d) Idiopathic thrombocytopenic purpura

Q57. A child presents with generalized edema, heavy proteinuria, low serum albumin, and hyperlipidemia. What is the most likely diagnosis?

- a) Acute nephritic syndrome
- b) Nephrotic syndrome
- c) Acute tubular necrosis
- d) Hemolytic uremic syndrome

Q58. At what age does a normally developing infant usually develop a social smile?

- a) 2 weeks
- b) 6 weeks



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- c) 6 months
- d) 1 year

Q59. A newborn develops poor feeding, vomiting, lethargy, and urine with a burnt sugar smell.

Which amino acids should be restricted in the diet?

- a) Phenylalanine and tyrosine
- b) Leucine, isoleucine, and valine
- c) Methionine and cysteine
- d) Lysine and tryptophan

Q60. A child with sickle cell disease has recurrent painful vaso-occlusive crises and episodes of acute chest syndrome. Which drug reduces these episodes by increasing fetal hemoglobin?

- a) Deferoxamine
- b) Hydroxyurea
- c) Desferrioxamine
- d) Vincristine

Q61. A 5-year-old child presents with hematemesis and splenomegaly. The child had exchange transfusion through the umbilical vein during the neonatal period. What is the most likely underlying cause?

- a) Splenic vein thrombosis
- b) Portal vein thrombosis
- c) Budd-Chiari syndrome
- d) Liver cirrhosis

Q62. A 3-year-old child with severe watery diarrhea has sunken eyes, lethargy, and poor oral intake. What is the best initial management?

- a) Oral zinc only
- b) Oral rehydration solution at home
- c) Intravenous fluid resuscitation
- d) Antidiarrheal medication



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- Q63. A 6-month-old infant has sudden flexion spasms occurring in clusters. EEG shows hypsarrhythmia. Which treatment is preferred?
- Carbamazepine
 - ACTH
 - Phenytoin
 - Ethosuximide
- Q64. A child has fever for 6 days, bilateral conjunctival congestion, strawberry tongue, cervical lymphadenopathy, and peeling of the skin over the fingers. Which cardiac complication is most important to recognize?
- Mitral stenosis
 - Coronary artery aneurysm
 - Ventricular septal defect
 - Dilated cardiomyopathy
- Q65. A 42-year-old man is admitted with severe dehydration after repeated episodes of vomiting and diarrhea during a summer wedding function. He has reduced urine output, dizziness, and low blood pressure. After IV fluids, his condition improves. The teacher explains that early ischemic injury in renal tubular cells may be reversible and is associated with failure of ion pumps, sodium-water influx, and cellular swelling. What change is seen?
- Apoptosis
 - Hydropic
 - Caseous
 - Dysplasia
- Q66. A 45-year-old man who drinks alcohol daily presents with mild right upper abdominal discomfort. He works late nights, often skips meals, and drinks with friends after work. Ultrasound shows enlarged fatty liver. The doctor explains that early alcoholic liver injury commonly causes accumulation of triglycerides within hepatocytes. Which change occurs?
- Steatosis
 - Metaplasia
 - Dysplasia
 - Atrophy



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- Q67. A 52-year-old man presents with fatigue, weight loss, night sweats, and fullness in the left upper abdomen. Examination shows massive splenomegaly. Blood counts reveal very high white cell count with cells of different stages of granulocytic maturation. The report mentions a translocation involving chromosomes 9 and 22. Which chromosome is formed?
- Philadelphia
 - Down
 - Patau
 - Turner
- Q68. All are true regarding primary open-angle glaucoma EXCEPT:
- The anterior chamber angle is open on gonioscopy.
 - It presents with sudden painful red eye and corneal edema.
 - Optic disc cupping may be seen.
 - Peripheral visual field loss may occur gradually.
- Q69. All are true regarding acute angle-closure glaucoma EXCEPT:
- It may present with severe ocular pain and headache.
 - Corneal edema may be present.
 - The pupil may be mid-dilated and poorly reactive.
 - The anterior chamber is typically deep.
- Q70. All are true regarding cataract EXCEPT:
- Cataract is opacity of the crystalline lens.
 - It commonly causes gradual painless diminution of vision.
 - It is primarily an opacity of the cornea.
 - Surgery involves removal of the cloudy lens and implantation of an intraocular lens.
- Q71. All are true regarding chalazion EXCEPT:
- It results from obstruction of a meibomian gland.
 - It is an acute painful infection of an eyelash follicle in every case.
 - It usually presents as a painless eyelid nodule.
 - Warm compresses may be used as conservative management.



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Q72. All are true regarding retinoblastoma EXCEPT:

- a) The most common presentation is painful blue sclera.
- b) Leukocoria may be seen.
- c) Strabismus may be a presenting feature.
- d) It is an intraocular malignancy of childhood.

Q73. All are true regarding refractive errors EXCEPT:

- a) Myopia causes difficulty seeing distant objects clearly.
- b) Hyperopia causes difficulty seeing near objects clearly.
- c) Astigmatism may cause distorted vision.
- d) Presbyopia is caused by corneal infection.

Q74. All are true regarding keratoconus EXCEPT:

- a) It is a progressive corneal ectatic disorder.
- b) It causes corneal thinning and steepening.
- c) It is an opacity of the crystalline lens.
- d) Corneal collagen cross-linking may help slow progression.

Q75. All are true regarding age-related macular degeneration EXCEPT:

- a) It affects the macula.
- b) It mainly causes peripheral vision loss and spares central vision.
- c) Drusen may be seen in dry AMD.
- d) Anti-VEGF injections are used in wet AMD.

Q76. All are true regarding bacterial keratitis EXCEPT:

- a) Topical steroids alone are the first-line emergency treatment.
- b) Contact lens wear is an important risk factor.
- c) Corneal ulceration and infiltrate may occur.
- d) Topical antibiotics are used in treatment.

Q77. All are true regarding pterygium EXCEPT:

- a) It is a fibrovascular growth of conjunctival tissue.
- b) It may encroach onto the cornea.



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- c) It is a malignant tumor arising from the crystalline lens.
- d) Ultraviolet exposure is an important risk factor.

Q78. Everything is true EXCEPT:

- a) Central perforation of the tympanic membrane is the typical diagnostic feature of otitis externa.
- b) Pain increases on movement of the pinna.
- c) The external auditory canal is inflamed and tender.
- d) Pseudomonas is an important causative organism.

Q79. Everything is true EXCEPT:

- a) Chronic ear discharge may occur.
- b) Conductive hearing loss may occur.
- c) Central perforation is usually considered unsafe disease.
- d) Tympanic membrane perforation is present.

Q80. Everything is true EXCEPT:

- a) Bone erosion may occur.
- b) It is completely harmless and never causes complications.
- c) Foul-smelling scanty ear discharge may occur.
- d) Attic perforation may be present.

Q81. Everything is true EXCEPT:

- a) Vertigo is positional.
- b) Dix-Hallpike test may be positive.
- c) Hearing loss is usually absent.
- d) Vertigo usually continues for many days without relief.

Q82. Everything is true EXCEPT:

- a) Unilateral sensorineural hearing loss may occur.
- b) It commonly arises from the facial nerve.
- c) Tinnitus may be present.
- d) It is also called vestibular schwannoma.



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Q83. Everything is true EXCEPT:

- a) Little's area is a common site of anterior epistaxis.
- b) Little's area is located on the anterior nasal septum.
- c) Little's area contains vascular anastomoses.
- d) Woodruff's plexus is the most common site of anterior epistaxis in children.

Q84. Everything is true EXCEPT:

- a) Nasal polyps are usually pale and edematous.
- b) Nasal polyps are usually insensitive to touch.
- c) Nasal polyps are usually painful, red, and bleed easily on touch.
- d) Nasal polyps may be associated with allergy.

Q85. All are true regarding chalazion EXCEPT:

- a) It results from obstruction of a meibomian gland.
- b) It is an acute painful infection of an eyelash follicle in every case.
- c) It usually presents as a painless eyelid nodule.
- d) Warm compresses may be used as conservative management.

Q86. All are true regarding trachoma EXCEPT:

- a) It is an infectious cause of blindness.
- b) It is associated with poor sanitation and overcrowding.
- c) It is caused by adenovirus.
- d) The SAFE strategy is used for control.

Q87. A 55-year-old patient presents with sudden onset flashes of light and floaters, followed by a curtain-like shadow descending over the visual field. Vision is progressively obscured in one eye. What is the most likely diagnosis?



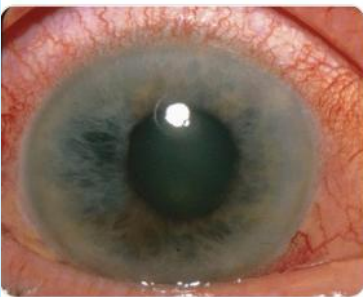
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- a) Retinal detachment (RD)
- b) Central retinal vein occlusion (CRVO)
- c) Cataract
- d) Uveitis

Q88. A middle-aged patient presents with sudden severe eye pain, headache, and seeing colored halos around lights. On examination, the eye is red with a mid-dilated, non-reactive pupil and decreased vision. What is the most likely diagnosis?



- a) Open-angle glaucoma
- b) Angle-closure glaucoma
- c) Uveitis
- d) Cataract

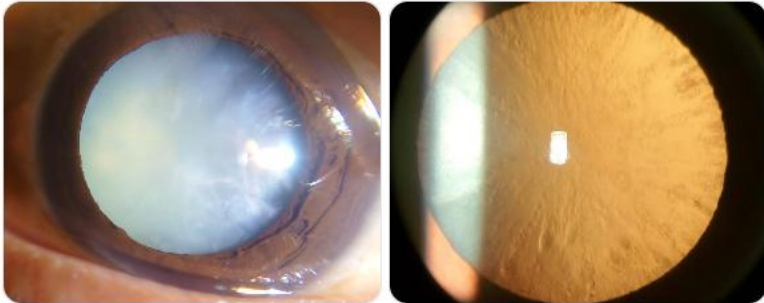
Q89. An elderly patient complains of gradual, painless decline in vision over months, along with increased glare, especially in bright light and during night driving. There is no pain or redness. What is the most likely diagnosis?





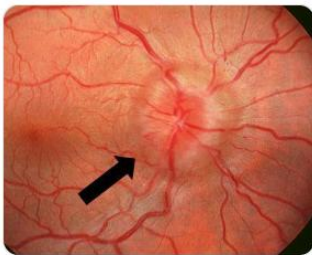
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- a) Cataract
- b) Glaucoma
- c) Retinitis pigmentosa (RP)
- d) Retinal detachment (RD)

Q90. A young adult presents with acute painful loss of vision, especially on eye movement. Examination reveals a relative afferent pupillary defect (RAPD) with decreased visual acuity. What is the most likely diagnosis?



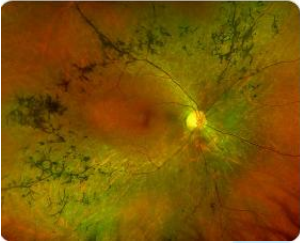
- a) Optic neuritis
- b) Glaucoma
- c) Retinal detachment (RD)
- d) Cataract

Q91. A young patient presents with difficulty seeing at night (nyctalopia) and progressive loss of peripheral vision leading to tunnel vision. Fundus examination may show bone-spicule pigmentation. What is the most likely diagnosis?



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- a) Retinitis pigmentosa (RP)
- b) Age-related macular degeneration (AMD)
- c) Cataract
- d) Glaucoma

Q92. A patient presents with acute painful swelling of the eyelid, associated with redness and tenderness near the lid margin. On examination, a localized tender nodule is noted. What is the most likely diagnosis?



- a) Chalazion
- b) Hordeolum
- c) Blepharitis
- d) Conjunctivitis

Q93. A young man presents with abdominal pain starting in the periumbilical region and later localizing to the right iliac fossa, associated with anorexia, low-grade fever, and vomiting. On examination, there is tenderness at McBurney's point with guarding and leukocytosis. What is the most likely diagnosis?



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- a) Acute cholecystitis
- b) Acute appendicitis
- c) Meckel diverticulitis
- d) Mesenteric adenitis

Q94. A 42-year-old obese woman presents with right upper abdominal pain, fever, and vomiting after a heavy meal. On examination, she has tenderness in the right hypochondrium and inspiratory arrest on deep palpation below the right costal margin.

Which is the most likely diagnosis?

- a) Acute hepatitis
- b) Acute appendicitis
- c) Acute cholecystitis
- d) Liver abscess

Q95. A 65-year-old man presents with progressive painless jaundice, pruritus, and clay-colored stools, along with significant weight loss. On examination, a palpable distended gallbladder is noted. What is the most likely diagnosis?



- a) Choledocholithiasis
- b) Hemolytic jaundice
- c) Carcinoma head of pancreas



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d) Acute viral hepatitis

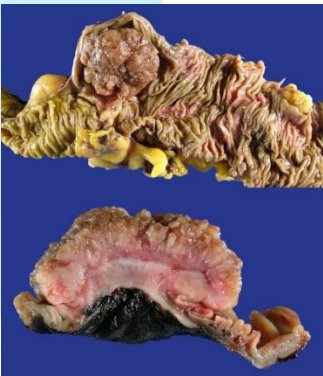
Q96. A 60-year-old man presents with painful irreducible groin swelling, vomiting, abdominal distension, and constipation. The swelling is tense and tender, with overlying erythema.

What is the most likely diagnosis?

- a) Reducible inguinal hernia
- b) Strangulated hernia
- c) Femoral aneurysm
- d) Hydrocele

Q97. A 58-year-old man presents with bleeding per rectum, tenesmus, mucus discharge, and altered bowel habits. Digital rectal examination reveals an irregular hard growth.

What is the most likely diagnosis?



- a) Ulcerative colitis
- b) Hemorrhoids
- c) Rectal carcinoma
- d) Proctitis

Q98. A patient after road traffic accident has paradoxical inward movement of a segment of the chest wall during inspiration and severe respiratory distress.

What is the most likely diagnosis?

- a) Tension pneumothorax



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- b) Flail chest
- c) Cardiac tamponade
- d) Pulmonary contusion alone

Q99. A young man suffers head injury and briefly loses consciousness, then regains it, and later deteriorates rapidly. CT scan shows biconvex hyperdense collection.

What is the most likely diagnosis?



- a) Subdural hematoma
- b) Epidural hematoma
- c) Subarachnoid hemorrhage
- d) Cerebral contusion

Q100. An elderly alcoholic presents with progressively worsening headache, confusion, and drowsiness after minor head trauma 2 weeks ago. CT scan shows a crescent-shaped collection crossing suture lines.

What is the most likely diagnosis?

- a) Epidural hematoma
- b) Subdural hematoma
- c) Extradural abscess
- d) Intracerebral hemorrhage

Q101. A 15-year-old boy presents with pain and swelling around the knee for 3 months. X-ray shows a sunburst appearance and Codman triangle in distal femur.

What is the most likely diagnosis?



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- a) Ewing sarcoma
- b) Osteosarcoma
- c) Giant cell tumor
- d) Osteoid osteoma

Q102. A 12-year-old boy presents with fever, pain, and swelling in the shaft of femur. X-ray reveals onion-skin periosteal reaction.

What is the most likely diagnosis?



- a) Osteosarcoma
- b) Chronic osteomyelitis
- c) Ewing sarcoma
- d) Osteoclastoma

Q103. A primigravida at 33 weeks gestation presents with headache and swelling of feet. Her blood pressure is 160/110 mmHg, and urine shows 2+ protein.

What is the most likely diagnosis?

- a) Gestational hypertension
- b) Chronic hypertension
- c) Pre-eclampsia
- d) Eclampsia



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Q104. A 25-year-old primigravida at 38 weeks presents with sudden leaking of clear fluid per vagina without labor pains. Speculum examination shows pooling of fluid. What is the most likely diagnosis?

- a) PROM
- b) PPROM
- c) Urinary incontinence
- d) False labor

Q105. During vaginal delivery, the fetal head delivers but retracts against the perineum. The shoulders fail to deliver.

What is the first-line maneuver?



- a) Fundal pressure
- b) McRoberts maneuver
- c) Vacuum extraction
- d) Forceps application

Q106. A woman in labor has CTG showing late decelerations after contractions. What is the most likely cause?

- a) Head compression
- b) Cord compression
- c) Placental insufficiency
- d) Normal physiological finding

Q107. A 22-year-old woman presents with 12 weeks amenorrhea, vaginal bleeding, excessive vomiting, and uterus larger than period of gestation. Ultrasound shows a snowstorm appearance.

What is the most likely diagnosis?



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- a) Missed abortion
- b) Molar pregnancy
- c) Ectopic pregnancy
- d) Placenta previa

Q108. A 58-year-old obese postmenopausal woman presents with bleeding per vaginum after 5 years of menopause.

What is the most likely diagnosis?

- a) Endometrial carcinoma
- b) Cervical ectropion
- c) Fibroid uterus
- d) Adenomyosis

Q109. A sexually active woman presents with painful swelling near the vaginal introitus, difficulty sitting, and local tenderness.

What is the most likely diagnosis?

- a) Bartholin cyst
- b) Bartholin abscess
- c) Vulval carcinoma
- d) Gartner duct cyst

Q110. A woman develops failure of lactation, amenorrhea, and weakness following severe postpartum hemorrhage.

What is the most likely diagnosis?

- a) Prolactinoma
- b) Sheehan syndrome
- c) PCOS
- d) Postpartum thyroiditis

Q111. A 14-year-old girl presents with cyclical lower abdominal pain and no menarche. On examination, a bulging bluish membrane is seen at the introitus.



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What is the most likely diagnosis?

- a) Turner syndrome
- b) Mullerian agenesis
- c) Imperforate hymen
- d) Gonadal dysgenesis

Q112. A 60-year-old multiparous woman complains of a mass descending per vaginum, worse on standing, along with backache and urinary symptoms.

What is the most likely diagnosis?

- a) Fibroid uterus
- b) Cervical polyp
- c) Uterovaginal prolapse
- d) Bartholin cyst

Q113. A 65-year-old man presents with progressive dyspnea, orthopnea, and paroxysmal nocturnal dyspnea. On examination, bilateral basal crepitations and an S3 gallop are present.

What is the most likely diagnosis?

- a) Bronchial asthma
- b) Right heart failure
- c) Left ventricular failure
- d) Constrictive pericarditis

Q114. A 32-year-old woman presents with exertional dyspnea and palpitations. Examination reveals a loud first heart sound, opening snap, and mid-diastolic rumbling murmur at the apex.

What is the most likely diagnosis?

- a) Mitral regurgitation
- b) Aortic stenosis
- c) Mitral stenosis
- d) Tricuspid stenosis



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Q115. A 40-year-old man with known valvular heart disease presents with fever, weight loss, splinter hemorrhages, and a new cardiac murmur.

What is the most likely diagnosis?

- a) Rheumatic fever
- b) Infective endocarditis
- c) Atrial myxoma
- d) Viral myocarditis

Q116. A 68-year-old hypertensive man presents with palpitations. Pulse is irregularly irregular, and no distinct P waves are seen on ECG.

What is the most likely diagnosis?

- a) Atrial flutter
- b) Sinus arrhythmia
- c) Atrial fibrillation
- d) Ventricular tachycardia

Q117. A 56-year-old diabetic man presents with fatigue, anorexia, pruritus, pedal edema, and pallor. Labs show high creatinine and small shrunken kidneys on ultrasound.

What is the most likely diagnosis?

- a) Acute kidney injury
- b) Chronic kidney disease
- c) Nephrotic syndrome
- d) Renal artery stenosis

Q118. A patient with cirrhosis develops confusion, altered sleep pattern, and asterixis after upper GI bleeding.

What is the most likely diagnosis?

- a) Meningitis
- b) Hepatic encephalopathy
- c) Alcohol withdrawal
- d) Stroke



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Q119. A 50-year-old man with alcohol use presents with abdominal distension, pedal edema, spider nevi, and splenomegaly.

What is the most likely diagnosis?

- a) Nephrotic syndrome
- b) Congestive cardiac failure
- c) Cirrhosis with portal hypertension
- d) Peritoneal tuberculosis

Q120. A 42-year-old man presents with fever, right upper quadrant pain, tender hepatomegaly, and raised alkaline phosphatase.

What is the most likely diagnosis?

- a) Acute cholecystitis
- b) Hepatic abscess
- c) Viral hepatitis
- d) Pancreatitis

Q121. A 46-year-old man with alcohol use presents with severe epigastric pain radiating to the back, vomiting, and abdominal tenderness. Serum lipase is elevated.

What is the most likely diagnosis?

- a) Perforated peptic ulcer
- b) Acute pancreatitis
- c) Myocardial infarction
- d) Acute gastritis

Q122. A young adult presents with recurrent episodes of bloody diarrhea, urgency, and tenesmus.

What is the most likely diagnosis?

- a) Irritable bowel syndrome
- b) Crohn disease
- c) Ulcerative colitis



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d) Amoebiasis

Q123. A 23-year-old woman presents with chronic diarrhea, abdominal pain, weight loss, and perianal fistula.

What is the most likely diagnosis?

- a) Ulcerative colitis
- b) Crohn disease
- c) IBS
- d) Celiac disease

Q124. A 30-year-old woman presents with fatigue, pallor, koilonychia, and pica. Peripheral smear shows microcytic hypochromic anemia.

What is the most likely diagnosis?

- a) Megaloblastic anemia
- b) Hemolytic anemia
- c) Iron deficiency anemia
- d) Aplastic anemia

Q125. A strict vegetarian presents with pallor, glossitis, paresthesia, and impaired vibration sense. Blood smear shows macro-ovalocytes and hypersegmented neutrophils.

What is the most likely diagnosis?

- a) Iron deficiency anemia
- b) Aplastic anemia
- c) Megaloblastic anemia
- d) Thalassemia trait

Q126. A patient presents with jaundice, pallor, splenomegaly, and raised reticulocyte count.

What is the most likely diagnosis?

- a) Iron deficiency anemia
- b) Hemolytic anemia



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- c) Aplastic anemia
- d) Anemia of chronic disease

Q127. A 38-year-old woman presents with symmetrical pain and swelling of MCP and PIP joints, morning stiffness lasting more than 1 hour, and fatigue.

What is the most likely diagnosis?

- a) Osteoarthritis
- b) Rheumatoid arthritis
- c) Gout
- d) SLE

Q128. A young woman presents with fever, joint pain, photosensitive rash over cheeks, oral ulcers, and proteinuria.

What is the most likely diagnosis?

- a) Rheumatoid arthritis
- b) Dermatomyositis
- c) Systemic lupus erythematosus
- d) Scleroderma

Q129. A 45-year-old woman presents with fatigue, weight gain, cold intolerance, constipation, dry skin, and delayed ankle jerk relaxation.

What is the most likely diagnosis?

- a) Hyperthyroidism
- b) Hypothyroidism
- c) Cushing syndrome
- d) Depression

Q130. A young patient with type 1 diabetes presents with vomiting, abdominal pain, dehydration, Kussmaul breathing, and fruity breath odor.

What is the most likely diagnosis?



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- a) Hyperosmolar hyperglycemic state
- b) Diabetic ketoacidosis
- c) Acute pancreatitis
- d) Lactic acidosis

Q131. A 34-year-old woman presents with fatigue, weight loss, postural dizziness, hyperpigmentation, and hyponatremia with hyperkalemia.

What is the most likely diagnosis?

- a) Cushing syndrome
- b) Addison disease
- c) Pheochromocytoma
- d) Hyperthyroidism

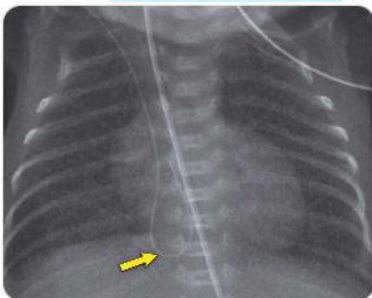
Q132. A 20-year-old student presents with fever, headache, vomiting, photophobia, and neck stiffness. Kernig sign is positive.

What is the most likely diagnosis?

- a) Migraine
- b) Meningitis
- c) Encephalopathy
- d) Subarachnoid hemorrhage

Q133. A preterm neonate develops tachypnea, grunting, and chest retractions soon after birth. Chest X-ray shows a diffuse ground-glass appearance.

What is the most likely diagnosis?





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- a) Transient tachypnea of newborn (TTN)
- b) Respiratory distress syndrome (RDS)
- c) Meconium aspiration syndrome (MAS)
- d) Pneumonia

Q134. A child presents with cyanosis and recurrent episodes of breathlessness, often relieved by squatting.
What is the most likely diagnosis?



- a) Atrial septal defect (ASD)
- b) Ventricular septal defect (VSD)
- c) Tetralogy of Fallot (TOF)
- d) Patent ductus arteriosus (PDA)

Q135. A child is found to have a loud pansystolic (holosystolic) murmur best heard at the left lower sternal border during routine examination.
What is the most likely diagnosis?

- a) Atrial septal defect (ASD)
- b) Ventricular septal defect (VSD)
- c) Patent ductus arteriosus (PDA)
- d) Tetralogy of Fallot (TOF)

Q136. A child is found to have a continuous “machinery-like” murmur on auscultation, best heard in the left infraclavicular area.
What is the most likely diagnosis?

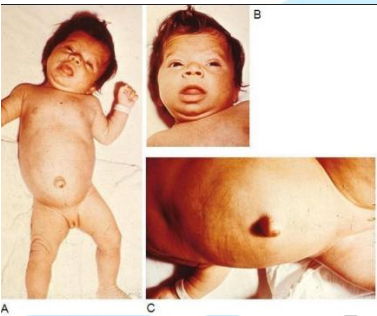
- a) Atrial septal defect (ASD)
- b) Ventricular septal defect (VSD)
- c) Patent ductus arteriosus (PDA)
- d) Tetralogy of Fallot (TOF)



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- Q137. A neonate presents with macroglossia, hypotonia, and prolonged neonatal jaundice. The baby is lethargic with poor feeding.
What is the most likely diagnosis?



- a) Down syndrome
- b) Congenital hypothyroidism
- c) Phenylketonuria (PKU)
- d) Galactosemia

- Q138. A 65-year-old woman falls on an outstretched hand and presents with pain and swelling at the wrist. On examination, a “dinner fork” deformity is noted.
What is the most likely diagnosis?



- a) Smith fracture
- b) Colles' fracture
- c) Scaphoid fracture
- d) Barton fracture



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Q139. A young man after a road traffic accident presents with a shortened limb that is adducted and internally rotated.

What is the most likely diagnosis?



- a) Anterior hip dislocation
- b) Posterior hip dislocation
- c) Neck of femur fracture
- d) Intertrochanteric fracture

Q140. A patient with tibial fracture develops severe pain, pain on passive stretch, tense swelling, and paresthesia.

What is the most likely diagnosis?



- a) Deep vein thrombosis
- b) Compartment syndrome
- c) Osteomyelitis
- d) Cellulitis



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Q141. A 32-year-old man presents with well-defined erythematous plaques covered with silvery scales over the extensor surfaces of elbows and knees. On scraping, pinpoint bleeding spots (Auspitz sign) are seen.

What is the most likely diagnosis?



- a) Lichen planus
- b) Psoriasis
- c) Tinea corporis
- d) Seborrheic dermatitis

Q142. A 40-year-old woman presents with itchy, purple, polygonal, flat-topped papules on the flexor surfaces of the wrist. Oral mucosa shows white lacy lines (Wickham striae).

What is the most likely diagnosis?



- a) Psoriasis
- b) Lichen planus
- c) Eczema
- d) Pityriasis rosea

Q143. A 50-year-old patient presents with flaccid bullae that rupture easily, leaving erosions. Nikolsky sign is positive. Oral mucosa is involved.



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What is the most likely diagnosis?



- a) Bullous pemphigoid
- b) Pemphigus vulgaris
- c) Dermatitis herpetiformis
- d) Stevens-Johnson syndrome

Q144. A 20-year-old student presents with intense itching, worse at night, with lesions in finger web spaces and wrists. Family members also have similar symptoms.

What is the most likely diagnosis?



- a) Pediculosis
- b) Scabies
- c) Eczema
- d) Contact dermatitis

Q145. A 35-year-old woman presents with persistent low mood, loss of interest, fatigue, poor concentration, and early morning awakening for the past 6 weeks. She has no history of mania.

What is the most likely diagnosis?

- a) Dysthymia
- b) Major depressive disorder
- c) Bipolar disorder



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d) Adjustment disorder

Q146. A 22-year-old man presents with auditory hallucinations, delusions of persecution, disorganized speech, and social withdrawal for 8 months.

What is the most likely diagnosis?

- a) Bipolar disorder
- b) Schizophrenia
- c) Delusional disorder
- d) Acute psychosis

Q147. A 28-year-old man presents with elevated mood, decreased need for sleep, increased talkativeness, grandiosity, and impulsive spending for 1 week.

What is the most likely diagnosis?

- a) Mania
- b) Hypomania
- c) Bipolar disorder
- d) Schizophrenia

Q148. A 30-year-old woman presents with recurrent sudden episodes of palpitations, sweating, chest tightness, and fear of dying, lasting 10–15 minutes, with no identifiable trigger.

What is the most likely diagnosis?

- a) Generalized anxiety disorder
- b) Panic disorder
- c) Phobia
- d) Somatization disorder

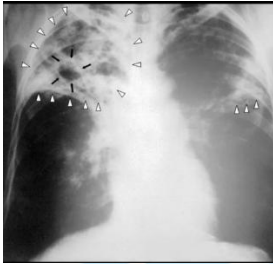
Q149. A patient presents with chronic cough, weight loss, and low-grade fever. Chest X-ray reveals cavitory lesions in the upper lobes.



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What is the most likely diagnosis?



- a) Pneumonia
- b) Tuberculosis
- c) Lung abscess
- d) COPD

Q150. A patient undergoing spinal anesthesia develops sudden hypotension and bradycardia. Cause?

- a) Hypovolemia
- b) High spinal block
- c) Allergic reaction
- d) Air embolism

ANSWER

1	c
2	c
3	d
4	c
5	d
6	a
7	b
8	a
9	b
10	c
11	b
12	a
13	a
14	b

15	b
16	c
17	c
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58	b
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61	b
62	c
63	b
64	b
65	b
66	a
67	a
68	b
69	d
70	c



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71	b
72	a
73	d
74	c
75	b
76	a
77	c
78	a
79	c
80	b
81	d
82	b
83	d
84	c
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86	c

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103	c

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142	b
143	b
144	b
145	b
146	b
147	c
148	b
149	b
150	b



NEXT

